

Hospitalization Risk Assessment

CCG: C-1007 (CCG)

MCG Health
Chronic Care
28th Edition

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Assessment

Case Management Evaluation

Q1. Case Manager: Does the patient have a primary care doctor?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Has the patient had either of the following in the past 6 months? (Check all that apply.)

- Unplanned hospitalization [P1]
- 2 or more emergency department visits [P1]
- Neither of the above
- Unknown [P1]

Q3. Case Manager: Does the patient have any medical risk factors? (Check all that apply.)

- AIDS [P1] HIV/AIDS - Self-Care [CCG](#)
- Anemia (including sickle cell) [P1] Sickle Cell Disease, Adult - Self-Care [CCG](#) Sickle Cell Disease, Pediatric - Self-Care [CCG](#)
- Asthma [P1] Asthma, Adult - Self-Care [CCG](#)
- Chronic obstructive pulmonary disease (COPD), symptomatic [P1] Chronic Obstructive Pulmonary Disease - Self-Care [CCG](#)
- Cystic fibrosis, symptomatic [P1] Cystic Fibrosis, Adult - Self-Care [CCG](#) Cystic Fibrosis, Pediatric - Self-Care [CCG](#)
- Depression [P1] Depression - Self-Care [CCG](#)
- Diabetes, uncontrolled or symptomatic [P1] Diabetes, Adult - Self-Care [CCG](#) Diabetes, Pediatric - Self-Care [CCG](#)
- End-stage liver disease [P1] Liver Disease - Self-Care [CCG](#)
- End-stage renal disease [P1] Kidney Disease - Self-Care [CCG](#)
- Frailty [P1] Frail Elderly - Self-Care [CCG](#)
- Heart disease, symptomatic [P1] Coronary Artery Disease - Self-Care [CCG](#)
- Heart failure [P1] Heart Failure - Self-Care [CCG](#)
- High-risk pregnancy [P1] High-Risk Pregnancy - Self-Care [CCG](#)
- Hypertension, uncontrolled or symptomatic [P1] Hypertension - Self-Care [CCG](#)
- Malnutrition [P1] Nutritional Management to Gain Weight [CCG](#)
- Metastatic solid tumor (eg, in breast or lung) [P1] Cancer - Self-Care [CCG](#)
- Polypharmacy [P1]
- Transplant (including pretransplant) [P1] Organ Transplant - Self-Care [CCG](#)
- Other, specify: _____ [P1]
- No
- Unknown [P1]

Q4. Case Manager: Does the patient have any psychosocial or socioeconomic risk factors? (Check all that apply.)

- Active behavioral health symptoms [P1]
- Active substance use [P1] Substance-Related Disorders - Self-Care [CCG](#)
- Adherence to care issues [P1] Medication Adherence [CCG](#)

- Affording basic living needs (eg, housing, food) [P1] Financial Status and Benefits [CCG](#) Community Resources [CCG](#) Food Insecurity [CCG](#)
- Affording medicine or supplies [P1] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P1] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Caregiver support insufficient [P1] Caregiver Resources Evaluation [CCG](#)
- Health literacy [P1] Health Literacy [CCG](#)
- Homeless [P1] Housing Insecurity [CCG](#) Community Resources [CCG](#)
- Home safety issues [P1] Safety [CCG](#)
- Social support insufficient [P1] Social Support [CCG](#)
- Transportation for care insufficient [P1] Transportation [CCG](#)
- Other, specify: _____ [P1]
- No
- Unknown [P1]

Q5. Case Manager: Is the patient postoperative with 1 or more of the following? (Check all that apply.)

- Blood glucose uncontrolled [P1] Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood pressure uncontrolled [P1] Blood Pressure Monitoring [CCG](#)
- Infection, active [P1]
- Obesity [P1] Obesity, Adult - Self-Care [CCG](#)
- Older than 85 years of age [P1]
- Ostomy, new [P1] Ostomy Management [CCG](#)
- Pain uncontrolled [P1] Pain Assessment [CCG](#)
- Smoker, active [P1] Tobacco Use [CCG](#)
- Wound, open [P1] Wound Management [CCG](#)
- Other, specify: _____ [P1]
- None of the above
- Unknown [P1]

Plan of Care

Problems

P1. Risk for hospitalization present

- **Goal:** Avoidable hospitalization risk factors evaluated and managed(1)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying and resolving modifiable readmission risks[NNNN](#)
 - **ASSIST:** Identifying and scheduling appointment with primary care provider(2)[N](#)
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool [SR](#) for more information.
 - **EDUCATE:** Potential risks for hospitalization and how to prevent, manage, or care for avoidable risks(3)
 - **EDUCATE:** Self-care plan appropriate for condition(4)
 - **EDUCATE:** When and how to get help for problems (eg, symptoms to seek emergency care for, when to call provider, use of community resources)
 - **COORDINATE:** Communication of care and transition information to primary provider(5)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Social worker referral, as needed(6)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** End-of-life or palliative care referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(7)(8)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(7)(9)(10)

- **COORDINATE:** Follow-up contact for evaluation of hospital risk factors, referral status, and need for further assistance.(2)(11)

Evidence Summary

Quality standards: The American Case Management Association's Transitions of Care Standards indicate that processes should be in place to identify patients at risk for care transition problems, patients' health risks should be assessed at each care episode or transition, and a care management plan should be established and communicated across care settings.(12)

Evidence: The literature discusses the creation and use of evidence-based risk models and algorithms, but results are mixed or inconclusive on effectiveness.(13)(14)(15)(16)

Evidence: Many studies report higher readmission rates in patients with functional problems, multiple comorbidities (and associated polypharmacy and self-care needs), and social determinants of health.(2)(17)(18)

Evidence: Several studies report that common clinical reasons for readmission are infection, gastrointestinal complications, bleeding, and fluid or electrolyte complications.(19)(20)

Evidence: Several studies found that risk factors for psychiatric 30-day readmission include substance abuse, major psychiatric diagnosis (eg, bipolar disorder, schizophrenia), comorbid medical diagnosis, and longer length of stay in the first hospitalization.(21)(22)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(23)(24)(25) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(2) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(26)(27)

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Chronic Care 28th Edition
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Last Update: 3/14/2024 3:11:59 AM
Build Number: 28.1.20240313235330.016020